

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

for the administration of Chloramphenicol eye drops and eye ointment for the treatment of Bacterial Conjunctivitis

Patient Group Direction, version 002, issued 11 September 2026. Supersedes Version 001, 1 November 2025.

Summary of NICE / NICE CKS Guidelines for Bacterial Conjunctivitis

Assessment and differential diagnosis of red eye

Bacterial conjunctivitis presents with conjunctival injection, purulent discharge, and possible chemosis. NICE guidance recommends assessment of red eye to exclude serious pathology including glaucoma, uveitis, corneal ulceration, and orbital cellulitis.

- Bacterial conjunctivitis: purulent discharge, unilateral or bilateral presentation common
- Risk factors: contact lens wear, poor hygiene, trauma, immunosuppression
- Exclude: severe eye pain, photophobia, reduced vision, corneal involvement, traumatic onset

Chloramphenicol as first-line topical antibiotic

Chloramphenicol is a broad-spectrum topical antibiotic effective against most bacterial pathogens causing conjunctivitis. It is well-tolerated with minimal systemic absorption.

- Available as 0.5% eye drops and 1% eye ointment
- Effective against *Staphylococcus aureus*, *Streptococcus* species, *Haemophilus influenzae*, *Enterobacteriaceae*
- Low risk of systemic toxicity at ophthalmic doses

Patient Group Direction

for the supply/administration of

Chloramphenicol 0.5% eye drops

for the treatment of

Bacterial Conjunctivitis

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
--	--

Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Bacterial conjunctivitis (acute bacterial infection of the conjunctiva)
Inclusion criteria	Clinical diagnosis of bacterial conjunctivitis: conjunctival injection, purulent discharge Adults and children aged 2 years and over Able to instil drops or have them instilled by carer
Exclusion criteria	Age less than 2 years (refer to GP) Hypersensitivity to chloramphenicol or any excipients Personal or family history of aplastic anaemia or other blood dyscrasias Concurrent bone marrow suppression or chemotherapy Severe eye pain, photophobia, or reduced vision (suggests more serious pathology - refer urgently to GP/eye casualty) Suspected corneal ulceration or abrasion (refer urgently) Suspected viral aetiology (herpes simplex - refer urgently)
Cautions	Contact lens wearers: advise removal during treatment and for 48 hours after completion; do not use drops while lenses are in situ Pregnancy and breastfeeding: use only if benefit outweighs risk; minimal systemic absorption expected Prolonged use (beyond 5 days) may be associated with very rare risk of aplastic anaemia; counsel patient to complete course but not to extend treatment without review

Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.
---	--

Details of the medicine

Name, form and strength of medicine	Chloramphenicol 0.5% eye drops
Legal category	P (Pharmacy supply)
Storage	Store below 25°C. Keep container tightly closed. Do not use after expiry date printed on the label.
Route/method of administration	Instillation into the conjunctival sac of the affected eye(s)
Dose and frequency	1 drop every 2 hours initially for 48 hours Then reduce to 1 drop four times daily (QDS) for 5 days total
Quantity to be administered and/or supplied	1 bottle, 10ml
Maximum or minimum treatment period	5 days total. Review if no improvement after 48 hours.
Adverse effects	Very common: transient stinging or irritation on instillation Common: mild conjunctival irritation, temporary blurred vision Rare: hypersensitivity reactions including conjunctival rash or urticaria Very rare: aplastic anaemia (associated with prolonged systemic use, not typical with topical ophthalmic use)

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear,

	legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)
--	--

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the chloramphenicol drops/ointment.
Follow-up advice to be given to patient or carer	Maintain excellent hand hygiene, washing hands before and after instillation/application Do not share towels, pillows, or cosmetics during treatment (infectious risk) Contact lens wearers must remove lenses during treatment and not reinert for 48 hours after completion If using drops, apply pressure to inner canthus (lacrimal duct area) for 1-2 minutes after instillation to reduce systemic absorption and improve local effect Symptoms should improve within 48 hours; if not, contact your GP to review diagnosis and consider alternative treatment Complete the full 5-day course even if symptoms resolve earlier (to prevent relapse) If symptoms worsen, or if you develop eye pain, photophobia, or vision changes, seek immediate medical advice (contact GP, call NHS 111, or attend eye casualty) Report any adverse reactions or unexpected effects to your GP or healthcare provider If pregnancy is discovered during treatment, inform your healthcare provider immediately (though topical use is low-risk)

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017
<https://www.nice.org.uk/guidance/mg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017
<https://www.nice.org.uk/guidance/mpg2/resources>
- NICE CKS Guidance: Bacterial Conjunctivitis
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date	Expiry date		
11/9/26	31/7/27		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025

Patient Group Direction
for the supply/administration of
Chloramphenicol 1% eye ointment
for the treatment of
Bacterial Conjunctivitis

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Bacterial conjunctivitis (acute bacterial infection of the conjunctiva) - particularly suitable for overnight application
Inclusion criteria	Clinical diagnosis of bacterial conjunctivitis Adults and children aged 2 years and over Able to apply ointment or have it applied by carer
Exclusion criteria	Age less than 2 years (refer to GP) Hypersensitivity to chloramphenicol or any excipients Personal or family history of aplastic anaemia or other blood dyscrasias Concurrent bone marrow suppression or chemotherapy

	Severe eye pain, photophobia, or reduced vision (suggests more serious pathology - refer urgently) Suspected corneal ulceration or abrasion (refer urgently) Suspected viral aetiology (refer urgently)
Cautions	Contact lens wearers: advise removal during treatment; ointment may damage or coat lenses Pregnancy and breastfeeding: use only if benefit outweighs risk Ointment may cause temporary blurred vision; warn patient not to drive or operate machinery Prolonged use (beyond 5 days) may be associated with very rare risk of aplastic anaemia
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Chloramphenicol 1% eye ointment
Legal category	P (Pharmacy supply)
Storage	Store below 25°C. Keep container tightly closed. Do not use after expiry date printed on the label.
Route/method of administration	Application into the conjunctival sac of the affected eye(s)
Dose and frequency	Apply to conjunctival sac at night (once daily) as adjunct to drops, or Apply four times daily (QDS) if patient unable to use or prefers drops alone
Quantity to be administered and/or supplied	1 tube, 4g
Maximum or minimum treatment period	5 days total (may be used in combination with drops or as alternative)
Adverse effects	Very common: transient blurred vision after application Common: mild irritation or grittiness Rare: hypersensitivity reactions Very rare: aplastic anaemia (associated with prolonged systemic use)

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
------------------------------	--

Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)
---------------------------	---

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the chloramphenicol drops/ointment.
Follow-up advice to be given to patient or carer	Maintain excellent hand hygiene, washing hands before and after instillation/application Do not share towels, pillows, or cosmetics during treatment (infectious risk) Contact lens wearers must remove lenses during treatment and not reinert for 48 hours after completion If using drops, apply pressure to inner canthus (lacrima duct area) for 1-2 minutes after instillation to reduce systemic absorption and improve local effect Symptoms should improve within 48 hours; if not, contact your GP to review diagnosis and consider alternative treatment Complete the full 5-day course even if symptoms resolve earlier (to prevent relapse) If symptoms worsen, or if you develop eye pain, photophobia, or vision changes, seek immediate medical advice (contact GP, call NHS 111, or attend eye casualty) Report any adverse reactions or unexpected effects to your GP or healthcare provider If pregnancy is discovered during treatment, inform your healthcare provider immediately (though topical use is low-risk)

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017
<https://www.nice.org.uk/guidance/mg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017
<https://www.nice.org.uk/guidance/mpg2/resources>
- NICE CKS Guidance: Bacterial Conjunctivitis
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date	Expiry date		
11/9/26	31/7/27		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025

Version and change record

Version: v002

Issued: 11 September 2026

Supersedes: Version 001, 1 November 2025

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

First reissue of this document.

Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Signed on behalf of Get Real Health

Version v002, 11 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		11 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		11 September 2026

Both authorising signatories reviewed this version together on 11 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.